PC-01 Pediatric Asystole / PEA

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | OCTOBER 26, 2026

Scope: Pulseless infant or child after the newly born period with asystole or pulseless electrical activity (PEA). Effective ventilation, high-quality CPR, and reversible-cause treatment are central interventions.

Field flow

1. Confirm	Apnea/pulselessness, DNR/MOST and obvious-death criteria; apply monitor/AED and check leads before treating apparent asystole.
2. CPR	100-120/min, depth at least one third chest AP diameter, full recoil, pauses under 10 seconds; 15:2 with two rescuers or 30:2 with one when no advanced airway.
3. Ventilate	Effective BVM first. With advanced airway: continuous compressions and 20-30 breaths/min; avoid hyperventilation.
4. IV/IO + epi	Obtain access without delaying CPR. Epinephrine 0.01 mg/kg IV/IO every 3-5 min.
5. Cause / ROSC	Treat reversible causes. If ROSC, use PC-08; if no ROSC, continue 2-minute cycles.

Provider-level actions

Level	Approved actions
EMT	Continuous high-quality CPR, AED/monitor, BVM oxygenation/ventilation, length-based system, and brief rhythm/pulse checks.
AEMT	All EMT care plus prompt IV/IO, epinephrine, glucose assessment, and crystalloid only when hypovolemia is suspected.
Paramedic	All prior care plus rhythm confirmation, waveform EtCO2, advanced reversible-cause treatment, and post-ROSC transition.

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SAFETY, MEDICATION, AND LINKED-PATHWAY REFERENCE

Safety and escalation

• Asystole/PEA is nonshockable; defibrillate only if the rhythm changes to VF/pulseless VT.

• Do not pause compressions for endotracheal intubation. IV/IO access and medication are secondary to CPR and ventilation.

• Do not routinely give calcium or sodium bicarbonate without a specific indication.

• Treat hypoxia, hypovolemia, acidosis, electrolyte/metabolic disturbance, hypothermia, tension pneumothorax, tamponade, toxins, and thrombosis.

Medication reference

Medication	Approved dose / safeguard
Epinephrine 0.1 mg/mL	0.01 mg/kg IV/IO as early as possible; max single dose 1 mg; repeat every 3-5 min.
Normal saline	10-20 mL/kg IV/IO only when hypovolemia is suspected; reassess perfusion and lungs after each bolus.

Linked pathways

AR-05 Pediatric Airway; AR-06 Pediatric Failed Airway; PC-02 Bradycardia / Poor Perfusion; PC-08 Post-Resuscitation; AO-02 Newly Born.

Source control

• This controlled direct source PDF replaces the prior source file and matches approved Claiborne EMS app content released October 26, 2026.

• Use current Claiborne standing orders, formulary, equipment authorization, and Medical Control direction whenever they govern a case-specific decision.